Peri-operative Management of Rheumatology Medications for Elective Surgery in North Devon.

Consensus reached by Rheumatologists at North Devon District General Hospital:

Issues to consider include:

i. Balance risk of infection v risk of flare.
ii. Other comorbidities
iii. Steroid use
iv. Type of surgery

These are general guidelines. If the anaesthetics or surgical team feel operative risks of infection are high rheumatology can be contacted to discuss on an individual case basis.

1. Subcutaneous Biologic Drugs		
Drug Name	Usual Regime	Recommendations
Adalimumab (Humira)	40 mg fortnightly sc	Omit one dose prior to surgery. Delay restarting treatment for at least one week post op, provided no perioperative infection AND wound is healed[1].
Etanercept (Enbrel)	50 mg weekly sc	Omit one dose prior to surgery. Delay restarting treatment for at least one week post op, provided no perioperative infection AND wound is healed[1].
Certolizumab	200mg fortnightly sc	Omit one dose prior to surgery. Delay restarting treatment for at least one week post op, provided no perioperative infection AND wound is healed[1].
Golimumab	50mg once monthly sc	Omit one dose prior to surgery. Delay restarting treatment for at least one week post op, provided no perioperative infection AND wound is healed[1].
Abatacept	125mg once weekly sc	Omit one dose prior to surgery. Delay restarting treatment for at least one week post op, provided no perioperative infection AND wound is healed[3].

2. *Intravenous Biologic Drugs*		
Drug Name	Usual Regime	Recommendations
Infliximab (Remicade)	3mg/kg every 8 weeks iv infusion	Minimum 4 week interval from last infusion. Recommence 4 weeks after surgery if no infection[2].
Tocilizumab (Roactemra)	8mg /kg every 4 weeks iv infusion	Delay surgery for at least two weeks from last infusion. Recommence 2 weeks after surgery if no infection[3].
Rituximab (Mabthera)	2 x1000mg 14 days apart not < each 6 months iv infusion	Delay surgery for 3 months following Rituximab infusions. Recommence 4 weeks after surgery if no infection. Measure immunoglobulins pre-operatively in patients at high risk of infection[3].
3. *Conventional DMARD's*		
Drug Name	Usual Regime	Recommendations
Methotrexate	7.5mg – 25mg once weekly oral or sc.	Continue throughout perioperative period. Monitor renal function post operatively. Risk of flare if MTX discontinued for >2 weeks[3,4].
Sulphasalazine	2-3g po daily in two divided doses	Hold day of surgery
Leflunomide	10-20 mg po once daily	Hold day of surgery
Azathioprine	100mg – 200mg po once daily	Hold day of surgery
Hydroxychloroquine (Plaquenil)	200-400mg po once daily	Continue throughout perioperative period
Mycophenolate Mofetil	500-1500mg po twice daily	Continue throughout perioperative period

References:

1. Saag KG et al. American College of Rheumatology 2008 recommendations for the use of non-biologic and biologic disease-modifying antirheumatic drugs in rheumatoid arthritis. Arthritis Rheum 2008; 59:762–784
2. Ding et al. BSR and BHPR rheumatoid arthritis guidelines on safety of anti-TNF therapies. Rheumatology (Oxford) 2010;49(11):2217-9
3. Goodman S et al. Perioperative Drug Safety in Patients with Rheumatoid Arthritis. Rheum Dis Clin N Am 2012. 38: 747-759
4. Visser K, et al. Multinational evidence-based recommendations for the use of methotrexate in rheumatic disorders with a focus on rheumatoid arthritis: Integrating systematic literature research and expert opinion of a broad international panel of rheumatologists in the 3E Initiative. Ann Rheum Dis 2009;68:1086-93.